

3rd week Wednesday case scenario

1-dr.

2-dr.

3-dr.

4-dr.

CASE 2

Mrs roza is a 40-year-old G3 P3, who presents complaining of painful periods. She was sterilised aged 35, and discontinued the Pill at that point. For the past four years, her periods have become increasingly painful; the pain starts a few days before onset of menstruation and resolves at the end of the period. Her cycles are 3/28, and there is no intermenstrual or postcoital bleeding. Her periods can be heavy occasionally, although this does not concern her as much as the pain. She does occasionally take time off her work as a career because of the pain.

2.1.From the history given, draw up a list of possible causes for the patient's symptoms.

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2.2.What additional questions would you like to ask the patient about her presenting complaint?

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2.3.You suggest to the patient that you wish to examine her. What signs are you going to look for, and how are these going to help you with your differential diagnosis?

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On examination, you find that Mrs roza is mildly overweight; her abdomen is soft with no obvious masses. On vaginal examination you find a normal-sized, fixed retroverted uterus, and there is tenderness in the posterior vaginal fornix. Both adnexa appear normal.

2.4.How might you investigate this patient? Please explain how the information from each test will help you.

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2.5.Suggest management strategies for dysmenorrhoea- try and think of the range of treatments available by underlying cause.

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2.6.Describe the mechanism of action of two different hormonal therapeutics you might use, their mode of administration, and their side effects

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